

[PATIENT_NAME]

Consult [DATE]
[FACILITY]

Provider: [PHYSICIAN] (Family Medicine)
Primary diagnosis: Preoperative clearance
Reason for Visit: Pre-op Exam

Progress Notes

[PHYSICIAN] (Physician) • Family Medicine

Subjective

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Patient ID: is a 72 y.o. female who presents for Pre-op Exam (Rt foot Sx with 1/21/26).

on

Right foot pain
Fell and broker her foot in high school
Started having pain 30 years ago
Did have some improvement with nerve blocks
Planning now to have a nerve release as pain is more severe

Surgery: Nerve release?
Surgeon:
Location: [FACILITY]
Date: 1/21/26
General Anesthesia

Denies previous complications with anesthesia in the past

Problem List

Patient Active Problem List

- Diagnosis
- Non-seasonal allergic rhinitis
 - Anxiety with depression
 - Hyperlipidemia
 - Hot flashes, menopausal
 - Post-traumatic osteoarthritis of right foot
 - Primary osteoarthritis, right hand
 - Other chronic pain
 - Atrial fibrillation (CMS/HCC) (HCC)

Surgical History

Past Surgical History:

Procedure	Laterality	Date
• CARPAL TUNNEL RELEASE	Right	
• EYE SURGERY		7/2023
• GASTRIC BYPASS		2014
• HYSTERECTOMY		2005
• KNEE SURGERY		2015

Family History

Family History

Problem	Relation	Name	Age of Onset
• Hypertension	Mother		
• Unspecified heart disease	Mother		
• Arthritis	Mother		
• Hypertension	Father	[PERSON_NAME]	
• Unspecified heart disease	Father	[PERSON_NAME]	
• Arthritis	Sister	[PERSON_NAME]	
• Hypertension	Sister	[PERSON_NAME]	
• Arthritis	Sister		
• Cancer	Sister		
• Hypertension	Sister		
• Hypertension	Sister	[PERSON_NAME]	
• Hypertension	Sister		
• Hypertension	Brother		
• Hypertension	Brother	[PERSON_NAME]	
• Type 2 diabetes mellitus without complications	Maternal Grandmother	[PERSON_NAME]	
• Diabetes	Maternal Grandmother	[PERSON_NAME]	
• Hypertension	Sister	[PERSON_NAME]	
• Arthritis	Sister	[PERSON_NAME]	
• Cancer	Sister		40
• Hypertension	Sister		

- Arthritis Sister
- Hypertensi Sister
on
- Asthma Sister [PERSON_N
AME] 15

- Rashes / Sister [PERSON_N
AME] 7
Skin
problems
 Dermatitis and — cellulitis at 50
- Hypertensi Brother
on
- Hypertensi Brother [PERSON_NAME]
on III

Social History

Social History

Tobacco Use

- Smoking status: Never
- Smokeless tobacco: Never

Substance Use Topics

- Alcohol use: Yes
 Comment: socially
- Drug use: No

Allergies

Allergies

Allergen	Reactions
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- Morphine Shortness of breath and Unknown
 Other Reaction(s): bradypnea

 Other Reaction(s): bradypnea

 Other Reaction(s): Not available

Medications Prior to Visit

Outpatient Medications Prior to Visit

Medication	Sig	Dispense	Refill
• celecoxib (CeleBREX) 50 mg capsule	TAKE 1 CAPSULE DAILY WITH FOOD AS NEEDED	90 capsule	0
• clobetasol (Temovate) 0.05 % external solution	Apply topically 2 times a day.		
• diclofenac (Voltaren) 75 mg EC tablet	Take 1 tablet twice a day by oral route as needed for 90 days.		
• estradiol (Estrace) 1 mg tablet	TAKE 1 TABLET DAILY	90 tablet	3
• FLUoxetine (PROzac) 20 mg tablet	TAKE 1 TABLET DAILY	90 tablet	1
• fluticasone (Flonase) 50 mcg/actuation nasal spray	Administer 2 sprays into each nostril 1 time each day.		

• HYDROcodone-acetaminophen (Norco) 7.5-325 mg tablet	Take 2 tablets by mouth if needed for severe pain.		
• ipratropium (Atrovent) 21 mcg (0.03 %) nasal spray	Administer 2 sprays into each nostril.		
• ketoconazole (NIZOral) 2 % shampoo	LATHER AND ALLOW TO SIT FOR SEVERAL MINUTES. USE TWICE PER WEEK		
• montelukast (Singulair) 10 mg tablet	TAKE 1 TABLET DAILY	90 tablet	3
• ProAir HFA 90 mcg/actuation inhaler	Inhale 2 puffs every 4 hours if needed for wheezing or shortness of breath.	18 g	2
• simvastatin (Zocor) 20 mg tablet	TAKE 1 TABLET DAILY IN THE EVENING	90 tablet	3
• gabapentin (Neurontin) 300 mg capsule	Take 1 capsule by mouth in the morning and 1 capsule before bedtime. (Patient not taking: Reported on [DATE])		
• FLUoxetine (PROzac) 20 mg tablet	Take 1 tablet by mouth 1 time each day. (Patient not taking: Reported on [DATE])	30 tablet	0

No facility-administered medications prior to visit.

Review of Systems

Constitutional: Negative for chills, fatigue, fever and unexpected weight change.

Respiratory: Negative for cough, shortness of breath and wheezing.

Cardiovascular: Negative for chest pain and palpitations.

Gastrointestinal: Negative for abdominal pain, diarrhea, nausea and vomiting.

Genitourinary: Negative for difficulty urinating.

Neurological: Negative for dizziness and headaches.

Objective

BP 111/71 | Pulse 54 | Temp 36.3 °C (97.4 °F) | Resp 18 | Ht 5' 4" | Wt 156 lb (70.8 kg) | SpO2 98% | BMI 26.78 kg/m²

Physical Exam

Vitals reviewed.

Constitutional:

Appearance: Normal appearance. She is well-developed.

HENT:

Head: Normocephalic and atraumatic.

Mouth/Throat:

Mouth: Mucous membranes are moist.

Pharynx: Oropharynx is clear.

Eyes:

Pupils: Pupils are equal, round, and reactive to light.

Neck:

Thyroid: No thyromegaly.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Pulses: Normal pulses.

Heart sounds: Murmur heard.

Pulmonary:

Effort: Pulmonary effort is normal.

Breath sounds: Normal breath sounds.

Musculoskeletal:

Cervical back: Neck supple. No tenderness.

Right lower leg: No edema.

Left lower leg: No edema.

Skin:

General: Skin is warm and dry.

Capillary Refill: Capillary refill takes less than 2 seconds.

Neurological:

Mental Status: She is alert. Mental status is at baseline.

Psychiatric:

Mood and Affect: Mood normal.

Behavior: Behavior normal.

Assessment & Plan

◆ Preoperative clearance

Cleared for surgery pending no abnormal labs.

Orders:

- CBC and differential
- Comprehensive metabolic panel
- ECG 12 lead

Right foot pain**Post-traumatic osteoarthritis of right foot****Abnormal EKG**